

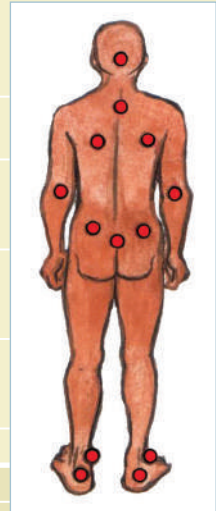
ROUTINE PALLIATIVE CARE

A patient can be given curative and palliative care at the same time. A doctor should confirm the patient needs palliative care:

- Patient is in bed or chair for 50% or more of the day or dependent on others for most care or has had 2 or more *unplanned* hospital admissions in past 6 months *and/or*
- Patient with advanced disease chooses palliative care only and refuses curative care *and/or*
- Patient with advanced disease not responding to treatment: heart failure, COPD, kidney or liver failure, cancer, HIV, TB, dementia or other progressive neurological disease.

Assess the patient needing palliative care

Assess	Note
Symptoms	• If pain: assess and advise ↪ 61. Give medication/s to treat pain ↪ 171. If new or sudden pain, temperature $\geq 38^{\circ}\text{C}$, tender swelling, redness or pus, also treat on symptom page. If no better or uncertain of cause, discuss. • If constipation, diarrhoea, nausea/vomiting, abdominal cramps, itchiness, acute anxiety or cough/difficulty breathing, manage ↪ 171. • If patient has difficulty sleeping ↪ 87. • Manage other symptoms as on symptom pages.
Side effects	• Ask about side effects from pain medication (see next page). • If on morphine, advise that nausea, confusion and sleepiness usually resolve after a few days. Check that patient is using regular laxative.
Mental health	• Ask if patient has persistent feelings of hopelessness or worthlessness? If yes ↪ 143. • If patient has suicidal thoughts or plans ↪ 83. • If low mood, stress or anxiety ↪ 86.
Chronic care	• Assess how much patient and family understand about the condition and ask what further information the patient and carer need. • Assess ongoing need for chronic care in discussion with patient and health care team. Consider which medication could be discontinued. • If known kidney failure with eGFR < 15, discuss.
Psychosocial	• Ask how patient is coping and what support and/or spiritual care is needed. • Ask how the carer/family are coping and what support they need now and in the future. • Ask about distressing social issues: problems with family relationships, finances, home care. If needed, refer patient's dependents and family members to social worker.
Dying	If known with terminal disease and deteriorating with ≥ 2 of: bed bound, decreased consciousness, only able to sip fluid, unable to take tablets, address patient's needs ↪ 172.
Mouth	Check oral hygiene and look for dry mouth, ulcers and thrush ↪ 35. If gum or tooth problem ↪ 36. If difficulty swallowing, discuss/refer.
Pressure sores	If patient is bedridden or in a wheel chair, check common areas for damaged skin (change of colour) and pressure sores (see picture). If patient has pressure ulcer/sore ↪ 75.



Advise the patient needing palliative care and his/her carer

- In a caring manner, explain the condition and prognosis. Explaining what is happening relieves fear and anxiety. Support the patient to give as much self care as able.
- Emphasise the importance of taking pain medication regularly (not as needed) and if using tramadol/morphine to use a laxative daily to prevent constipation.
- Refer patient and carer to available community health worker, social worker, physiotherapist, counsellor, spiritual counsellor, support group ↪ 178 Deal with bereavement issues ↪ 86.
- Prevent mouth disease: brush teeth and tongue regularly using toothpaste or dilute bicarbonate of soda. Rinse mouth with $\frac{1}{2}$ teaspoon of salt in 1 cup of water after eating and at night.
- If bedridden or in wheelchair, prevent pressure ulcers: wash and dry skin daily. Keep linen dry. Move (lift, avoid dragging) patient every 1-2 hours if unable to shift own weight. Look daily for skin colour changes (see picture).
- If bedridden, prevent contractures: at least twice a day, gently bend and straighten joints as far as they go. Avoid causing pain. Massage muscles.
- The patient's appetite will get less as s/he gets sicker. Offer small meals frequently and allow the patient to choose what s/he wants to eat from what is available and encourage fluid intake.
- Discuss the plan for caring for the patient. Advise whom to contact when pain or other symptoms get severe. Discuss advance-care plans and preferences. Document decisions.
- Educate the carer to recognise signs of deterioration and impending death: s/he may be less responsive, become cold, sleep a lot, have irregular breathing, and will lose interest in eating.